



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سامية جمال احمد محمد
المحافظة : الجيزة
الحى / المدينة : العياط
العنوان : =
رقم البطاقة : 28504302100202 رقم التليفون : 01126277046
تاريخ تسجيل الإستمارة : 25-09-2026
العمر (تاريخ الميلاد) : 30-04-1985 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: سامية جمال احمد محمد National ID: 28504302100202	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



١١٠٣٥٧٧

Patient's Name: samia gamal ahmed mohamed
 Date: 2026-04-05
 Referred by: Prof.Dr. ct cap iv
 Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of managed left breast cancer, compared to the last CT report dated 10/6/2021, the current examination revealed

I- CHEST:

- Left MRM showed prominent axillary lymph nodes overlying granulation tissue measures about 1.1x1 cm for further sonomammography
- Left upper and middle lobar reticulations likely post radiation sequel.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.



II- ABDOMEN AND PELVIS:

- Average sized liver showing fatty parenchymal attenuation. Newly developed bilobar multiple hypodense focal lesions, the largest is seen in segment VIII measure about 1.8x1.25 cm for further triphasic CT/Dynamic MRI
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen (splenule about 9 mm) , pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric para-aortic lymphadenopathy.
- No ascites.

Giza medical syndicate
Ayat central hospital
Radiology department

Name: Samia Gamal Ahmed

Date: 18-4-2026

Pelvi-Abdominal Ultrasound Examination reveals

✓ Liver :

- Average in size=15.5cm and bright echo pattern.
- No focal lesions were detected.
- Portal vein is not dilated.
- CBD is not dilated

✓ GB :-

- Average size, no evidence of calculi seen inside.
- Normal wall thickness.

Spleen :

- of average size=11.5cm with no focal lesions. Splenic vein is not dilated.

✓ Kidneys :

- Are of average in size
- Average parenchymal thickness and echogenicity with good cortico medullary differentiation
- Bilateral renal gravels with LT small upper calyx stone measures 4.5mm and LT mild hydronephrosis and dilated proximal ureter with no evidence of obstructive stones.

✓ urinary bladder :-

- Is of average distention and normal wall thickness.
- No stones , masses or diverticular outpouchings
- Clear content

✓ The mid line structure

- Appear normal

✓ No ascites

✓ No evidence of bowel dilatations or masses seen

conclusions :-

- Bilateral renal gravels with LT small stone and LT mild hydronephrosis, for lab and clinical correlation.

Dr. Ahmed Darka

مستشفى
روستاني
جولف

أ.د. 9-9



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المنشأة المصرية
مصر

09/09/2026

MDT FORM

Name:	Samia Gamal Ahmed Mohamed	MRN:	4458
Age:	41 Y	ID:	28504302100202
Menopausal Sta		Phone:	01113020477
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	nad	Reason Of MDT:	visceral crisis

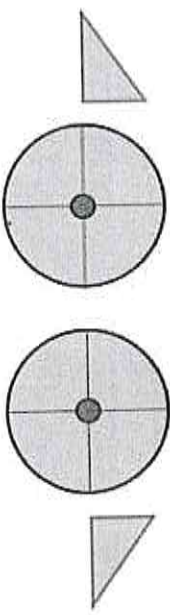
Clinical Data:

BX: R

ER: PR: HER2: KI67:

Sonomammography:

Mets Workup:



History:

A 40-year-old female, a case of left breast cancer, diagnosed as (IDC), grade 2, initially presented as cT4N1. She received NACT in the form of 4 cycles of AC, followed by 4 cycles of (docetaxel), then underwent left MRM in February 2022. Final pathology showed ypT2N2. She was started on adjuvant tamoxifen with (Zoladex) since March 2022, and was shifted to an AI (Femara) 3 months ago + zoladex

T N M

CT 04/2026 >> newly developed hepatic focal lesions largest 1.8x1.25 , L4
lytic lesion and S1 sclerotic lesion

Bx from HFLs (1-7-2026) > of breast origin , ER 4/8 PR 0/8 her2 +1 KI 67 %
20 %

was planned for Faslodex Ibrance > received 1 cycle
but PR now in visceral crisis bil 11/9 >> Ibrance holded and now only on
faslodex

PAUS (1-9-2026) : minimal IHBD
for possibility of urgent Cth

Recommendations:

--

MDT Outcome & Plan

In view of impending visceral crisis, for:

1. Taxol/Carbo weekly urgently
2. PDLL testing.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد يوسف	د. لقيه عصاري				

أ.د. 9/9/26

Patient Name	سالمه جمال احمد محمد	Case No.	264563
Sex/Age	Female 41 Y	Patient No.	20356
Registered Date	04/6/2026 11:54:54 AM	Referred By Dr.	Interventional Radiology
Validation Date	01/7/2026 03:06:25 PM	Branch	Gustave Roussy
Reporting Date	01/7/2026 03:34:35 PM	Pathology No.	1751/26

Pathology report

Clinical Data: Hepatic focal lesion, in a known case of left breast cancer as the patient underwent left MRM since March 2022.

Nature of Specimen: Hepatic focal lesion, Tru-cut biopsy, collected on 04/06/2026, at 10:30 am, cold ischemia time: 20 seconds.

Gross Examination:

- Fragmented grey white tissue cores, ranging in size between 0.2 cm & 0.5 cm long. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

* Histopathological examination reveals cores of hepatic tissue showing infiltration by tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in clusters with poor tendency to tubular formation amid desmoplastic stroma. There is focal necrosis.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Moderate nuclear staining in 20% of tumor cells (score 4/8).
- PR: Negative nuclear staining in tumor cells (score 0/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 20%.
- GATA-3: Positive in tumor cells

Diagnosis:

Hepatic focal lesion, Tru-cut biopsy:

Findings are consistent with metastatic carcinoma of breast origin.

IPT:

- ER: Positive (score 4/8).
- PR: Negative (score 0/8).
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**** Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Gamal Abde El Aty.
Dr. Walaa Ibrahim Chonaim



خريطة رقم (4) - القدر الأول - مبنى رقم (987) - مركز فحص القبرلقول - مصر القبرلقول - القبرلقول - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Msr El Qaderma - Cairo, Egypt

Patient's Name: samia gamal ahmed

Date: 2026-09-01

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt

ABDOMEN AND PELVIC ULTRASONOGRAPHIC SCANNING

REVEALED:

Liver is of enlarged sized measuring 18.9 cm in RT MCL, shows regular outline and homogenous echopattern. With multiple bilobar hypoechoic focal lesions the largest measuring 2.9x3 cm, Minimal intrahepatic biliary dilatation are seen.

Portal vein: is patent.

Gall bladder shows multiple tiny stones and mud (hepatization) CBD is seen mildly dilated 9 mm

Spleen is average sized and showing homogenous echopattern;

Pancreas and para-aortic region are concealed by gases.

Both kidneys show bilateral mild backpressure changes otherwise are of average size and shows homogenous echopattern. No stones or masses are seen.

Uterus is of average size, having regular well defined outlines. The myometrium is of average thickness, and normal homogenous echopattern showing no masses.

No ovarian or adenexal solid or cystic lesions.

The urinary bladder shows normal wall thickness and clear contents. No stones, polyps or masses, or diverticular.

Mild ascites.

CONCLUSION:-

- ***Hepatomegaly with multiple hypoechoic focal lesion ? metastatic for dynamic MRI.***
- ***GB hepatization with mildly dilated CBD and minimal IHB RDs***
- ***Bilateral hydronephrosis for CTUT***
- ***Mild Ascites***

Much obliged,
DR. Hend Ashraf

مستشفى
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لدعم صحة المرأة المصرية

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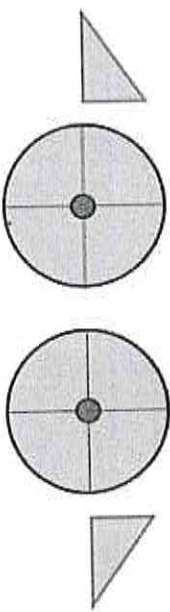
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